Capecitabine

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Capecitabine · active treatment cycle

Active medicines: Capecitabine

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Metoclopramide	10 mg PO up to three times daily as required	Only when needed, as prescribed One option for oral minimal-low risk; check renal function, contraindications, interactions and duration. Do not add all alternatives concurrently.

Additional phase-specific support

- The team agrees a rescue medicine if needed; routine daily anti-sickness prevention is not automatically required for this oral low-risk schedule.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.
- Where prescribed under the capecitabine source: loperamide 4 mg initially then 2 mg after each loose stool, maximum 16 mg/day; follow the written local escalation plan. Do not copy the irinotecan high-dose schedule.
- Swallow capecitabine whole with water and food or within 30 minutes afterwards. Do not crush or cut tablets; missed toxicity doses are not replaced.
- Interrupt immediately for moderate/severe toxicity and obtain oncology advice. Grade 2 or worse dehydration requires interruption until rehydrated and the cause is controlled; do not restart automatically after the planned break.
- Record previous dose reductions before prescribing the next cycle. A table percentage must not automatically re-escalate a toxicity-reduced dose. Partial-dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) starting-dose titration is a separate specialist pathway.
- Confirm blood count, renal and liver profile before every cycle; DPD testing precedes first treatment unless already tested. Plan international normalised ratio (blood-clotting test) (INR) checks if a vitamin K antagonist is used.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00216 v8 · NCCP Classification V6 (2025), oral minimal-to-low risk Table 5

Sources checked 2026-11-05 · SACTCheck v0.84.36 · Regimen-level drafts; prescriber and oncology-pharmacy verification required